

[DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity — monthly report

Project: [DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity
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Prepared with DonorDesk

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Executive Summary

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In April 2026, the USAID-funded Turkana Maternal, Newborn and Child Health (MNCH) Strengthening Activity, implemented by GEC with the Lodwar Community Health Alliance (LOCHA) in Turkana County, Kenya, recorded its strongest month to date. The period produced 8 activity records and 16 verified indicator results, and immunization and antenatal numbers rose well above March.

The project trained 26 health workers in Emergency Obstetric and Newborn Care (EmONC), Integrated Management of Childhood Illness (IMCI) and essential newborn care (15 female, 11 male), against a life-of-activity target of 120, up from 14 in March. It delivered essential equipment kits to 3 of 14 facilities, completed 14 mentorship visits, trained and kitted 60 community health volunteers (38 female, 22 male), and reached 2600 people with community health education. Facilities recorded 640 first antenatal care visits, 280 skilled deliveries and 420 children under one receiving DTP3/Penta3 through 28 outreach sessions. Service indicators remained below expectation: four or more antenatal visits stood at 32% against a 45% target, skilled delivery at 46% against 60%, and 72% of facilities submitted Kenya Health Information System (KHIS) reports on time against a 90% target.

Long-rains flooding on the Turkwel and Kerio rivers cut the Kalokol and Lowarengak roads for nine days, so two outreach sites could not be visited; routes were re-planned and vouchers prioritised for women with danger signs. In May, the project will continue training cohorts, deliver the next equipment batch, and hold monthly mentorship and data reviews.

Progress Against the Work Plan

Progress by Intermediate Result (IR), April 2026.

IR1: Health worker capacity and facility systems.

Project clinical trainers and nurse mentors trained 26 health workers (15 female, 11 male) in EmONC, IMCI and essential newborn care at Kalokol Health Centre on 9 April 2026 and completed 14 facility clinical mentorship visits across the 14 supported facilities. The logistics team delivered and installed essential MNCH equipment kits at 3 facilities on 14 April, with handover certificates signed by facility in-charges. A monthly data review and KHIS data quality audit at Lodwar on 25 April brought together 28 facility in-charges and records officers, and 72 % of supported facilities submitted KHIS monthly reports on time.

IR2: Service delivery.

Supported facilities recorded 640 first ANC visits, 280 skilled deliveries and 22 women referred for emergency obstetric care through the transport voucher scheme, alongside 28 outreach immunization sessions that delivered DTP3/Penta3 to 420 children under one year (210 female, 210 male). ANC4+ coverage reached 32 %, skilled delivery coverage 46 % and DTP3/Penta3 coverage 66 %. These services were delivered with county health staff, facility teams and community health volunteers at facilities in Turkana Central, Turkana South and Loima sub-counties, including Kerio and Lowarengak health centres.

IR3: Community engagement and data systems.

The project trained and kitted 60 CHVs (38 female, 22 male) on the community health package, iCCM, MUAC screening and danger-sign counselling. CHVs and community mobilisers reached 2600 people (1612 female, 988 male) with health education through dialogue days, household visits and radio sessions, screened 1300 children aged 6-59 months for acute malnutrition using MUAC (663 female, 637 male) and referred 90 children with acute malnutrition to treatment.

Indicator performance table

Indicator	Month result	Cumulative result	Life-of-activity target	Percent of target
ANC4+ coverage (HL-OC1a)	32 %	32 %	45 %	71.1 %
Skilled birth attendant coverage (HL-OC1b)	46 %	46 %	60 %	76.7 %
DTP3/Penta3 coverage under 1 year (HL-OC1c)	66 %	66 %	80 %	82.5 %
Health workers trained in EmONC, IMCI or essential newborn care (HL-1.1a)	26	40	120	21.7 %
Facility clinical mentorship visits (HL-1.1b)	14	22	84	16.7 %
Facilities equipped with essential MNCH kit (HL-1.2a)	3	3	14	21.4 %
First ANC visits (HL-1.3a)	640	1020	4200	15.2 %
Skilled deliveries (HL-1.3b)	280	440	2100	13.3 %
Emergency obstetric care transport voucher referrals (HL-1.3c)	22	32	180	12.2 %
Children under 1 year receiving DTP3/Penta3 (HL-1.4a)	420	710	3300	12.7 %
Outreach immunization sessions (HL-1.4b)	28	46	180	15.6 %
Facilities submitting KHIS reports on time (HL-1.5a)	72 %	72 %	90 %	80 %
CHVs trained and kitted (HL-2.1a)	60	100	220	27.3 %
People reached with community health education (HL-2.2a)	2600	4400	18000	14.4 %
Children 6-59 months screened with MUAC (HL-2.3a)	1300	2200	9000	14.4 %
Children with acute malnutrition referred to treatment (HL-2.3b)	90	150	650	13.8 %

Indicators behind plan and corrective action.

All indicators remain below their life-of-activity targets. The project records that some counts, such as skilled deliveries, are still behind their planned monthly share because roads were cut during the rains and two outreach sites were missed: long-rains flooding on the Turkwel and Kerio rivers cut the Kalokol and Lowarengak roads for nine days, and a national Penta vaccine shortage lasted one week. Corrective actions recorded for April were the re-planning of outreach routes around the flooded roads and the recall of children missed during the stock-out for vaccination in the following weeks. Intermittent internet and power outages delayed KHIS uploads at remote facilities; the project will follow up on late-reporting facilities through the monthly data review.

Rationale.

The April plan prioritised training, mentorship, equipment delivery and community-level services, and the recorded results show delivery at pace: trainings and mentorship ran at full pace, the first equipment kits reached three facilities, and immunization and antenatal numbers rose above March (640 first ANC visits up from 380; 420 children vaccinated with DTP3/Penta3 up from 290). The fixed outreach calendar shared with community health volunteers helped caregivers find outreach sessions even when routes changed, which the project notes as a lesson for the next period.

Planned activities for the next reporting period.

The project will continue training cohorts of health workers and hold monthly mentorship visits, deliver the next batch of essential MNCH equipment kits to the remaining facilities, continue outreach ANC clinics and voucher reimbursements to transporters, continue outreach immunization sessions and monthly cold-chain checks, train the next CHV cohort and supervise those already trained, continue dialogue days and radio messages, continue monthly MUAC screening and track referral completion, and follow up on late-reporting

